

PACKAGE INSERT

BETAMETHASONE CREAM 0.1% W/W

Composition : Betamethasone 17-Valerate
equivalent to Betamethasone 0.1%w/w

Product Description: A white and smooth cream.

Pharmacodynamics:

Topical corticosteroids, such as betamethasone valerate, are effective in the treatment of corticosteroid-responsive dermatoses primarily because of anti-inflammatory, anti-pruritic and vasoconstrictive actions. Its exact mechanism of action is not known. It appears to stabilize the membranes of lysosomes, thereby promoting the containment not only of cathepsins especially but also of other enzymes within the lysosomal sac.

Pharmacokinetics:

The extent of percutaneous absorption of topical corticosteroids is determined by many factors including the vehicle, the integrity of the epidermal barrier, and the use of occlusive dressings. Topical corticosteroids can be absorbed from normal intact skin. Inflammation and/or other disease processes in the skin will increase percutaneous absorption. Occlusive dressings substantially increase the percutaneous absorption of topical corticosteroids.

Once absorbed through the skin topical corticosteroids are handled through pharmacokinetic pathway similar to systemically administered corticosteroids. Corticosteroids are bound to plasma proteins in varying degrees. Corticosteroids are metabolised primarily in the liver and are then excreted by the kidneys. Some of the topical corticosteroids and their metabolites are also excreted into the bile.

Indication:

It is indicated for relief of the inflammatory and pruritic manifestations of corticosteroid-responsive dermatoses, eg psoriasis and allergic and inflammatory dermatitis.

Recommended Dosage:

A thin film of the cream is applied to the affected skin area one to three times a day. Dosage once or thrice a day is often effective.

Route of Administration:

Topical

Contraindications:

It is contraindicated in patients who are hypersensitive to betamethasone valerate, or to other corticosteroids, or to any ingredients in this preparation.

Warnings and Precautions:

Patients receiving a large dose of a potent topical steroid applied to a large surface area or prolonged use or under an occlusive dressings should be evaluated periodically for evidence of HPA axis suppression by using the urinary free cortisol and ACTH stimulation tests. If HPA axis suppression is noted, an attempt should be made to withdraw the drug, to reduce the frequency of application, or to substitute a less potent steroid. Recovery of HPA axis function is generally prompt and complete upon discontinuation of the drug. Infrequently, signs and symptoms of steroid withdrawal may occur, requiring supplemental systemic corticosteroids.

If irritation or sensitization develops, topical corticosteroids should be discontinued and appropriate therapy instituted. In the presence of dermatological infections, an appropriate anti-fungal or antibacterial agent should be instituted.

If a favorable response does not occur promptly, the corticosteroid should be discontinued until the infections has been adequately controlled. This preparation is for external use only. Avoid introduction of the cream into the eye.

If a reaction suggesting sensitivity or chemical irritation should occur, use of this medication should be discontinued.

Visual disturbance

Visual disturbance may be reported with systemic and topical corticosteroid use. If a patient presents with symptoms such as blurred vision or other visual disturbances, the patient should be considered for referral to an ophthalmologist for evaluation of possible causes which may include cataract, glaucoma or rare diseases such as central serous chorioretinopathy (CSCR) which have been reported after use of systemic and topical corticosteroids.

Interactions with Other Medicaments:

No known drug interaction unless systemic absorption of topical corticosteroids occur.

Pregnancy and Lactation:

Topical corticosteroids should be used during pregnancy only if the potential benefit justifies the potential risk to the fetus. Drugs of this class should not be used extensively on pregnant patients, in large amounts, or for prolonged periods of time.

It is not known whether topical administration of corticosteroids can result in sufficient systemic absorption to produce detectable quantities in breast milk.

Systemically administered corticosteroids are secreted into breast milk in quantities not likely to have a deleterious effect on the infant. Nevertheless, caution should be exercised when topical corticosteroids are prescribed for nursing woman.

Side Effects:

The following local adverse reactions have been reported with topical dermatological corticosteroids especially under occlusive dressings: burning, itching, irritation, dryness, folliculitis, hypertrichosis, acneiform eruptions, hypopigmentation, perioral dermatitis, allergic contact dermatitis, maceration of the skin, secondary infection, skin atrophy, striae and milium. Systemic absorption of topical corticosteroids has produced reversible hypothalamic-pituitary-adrenal (HPA) axis suppression, manifestations of Cushing's syndrome, hyperglycemia and glucosuria in some patients. Vision, blurred.

Symptoms and Treatment of Overdose:

Excessive prolonged use of topical corticosteroids can suppress pituitary-adrenal function resulting in secondary adrenal insufficiency.

Appropriate symptomatic treatment is indicated. Acute hypercorticism symptoms are virtually reversible. Treat electrolyte imbalance, if necessary. In cases of chronic toxicity, slow withdrawal of steroids is advised.

Effects on Ability to Drive and Use Machine:

Not applicable.

Storage Conditions:

Store at or below 30°C. Protect from light. Keep the container well closed.

Pack Size:

A tube of 15g and 30g

A box of 50 x 15g/tube.

A jar of 100g.

Pack Size (export only): A jar of 400g and 500g

Shelf-life:

3 years.

FURTHER INFORMATION CONCERNING THIS DRUG CAN BE OBTAINED FROM YOUR FAMILY PHYSICIAN / LOCAL GENERAL PRACTITIONER / PHARMACIST.

Manufacturer & Product Registration Holder:

SUNWARD PHARMACEUTICAL SDN. BHD.

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